

CITYCARE GENERAL HOSPITAL

PATIENT DISCHARGE STANDARD
OPERATING PROCEDURE

Safe & Effective Inpatient Discharge Management

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1. PURPOSE & SCOPE

This SOP defines the standards and processes for safe, timely, and well-coordinated patient discharge from CityHospital inpatient wards. A well-executed discharge reduces readmission rates, ensures continuity of care, and supports patient self-management after leaving hospital.

- Initiate discharge planning from day one of admission for all inpatients.
- Ensure all clinical, social, and administrative discharge criteria are met before patient leaves.
- Provide comprehensive written and verbal discharge instructions to patient and family.
- Facilitate smooth handover of care to community healthcare providers and general practitioners.
- Achieve target discharge time of 11:00 hrs for all planned discharges to optimise bed flow.

2. DEFINITIONS & ABBREVIATIONS

- **Planned Discharge:** Physician-anticipated discharge on a known date.
- **Unplanned Discharge:** Patient leaves hospital earlier than anticipated by clinical team.
- **AMA Discharge:** Patient leaves against medical advice despite clinical recommendation to stay.
- **Discharge Criteria:** Clinical and social benchmarks a patient must meet before safe discharge.
- **Transition of Care:** Transfer of clinical responsibility from hospital to community or another facility.
- **DAMA:** Discharge Against Medical Advice.
- **GP:** General Practitioner (primary care physician).
- **SBAR:** Situation–Background–Assessment–Recommendation handover communication framework.
- **Medication Reconciliation:** Process of comparing inpatient medications to discharge prescriptions.
- **EDD:** Estimated Discharge Date — set at admission and updated at each ward round.
- **MDT:** Multidisciplinary Team — includes physicians, nurses, physiotherapists, social workers, and pharmacists.

3. ROLES & RESPONSIBILITIES

Role	Department	Responsibilities
Attending Physician	Relevant Specialty	Discharge decision, discharge summary, follow-up plan
Discharge Coordinator	Discharge Office	Coordinate MDT discharge plan, book follow-up appointments
Ward Nurse	All Wards	Patient education, medication counselling, discharge documentation
Pharmacist	Pharmacy	Discharge prescription preparation, medication counselling, reconciliation
Social Worker	Social Services	Home support, safeguarding referrals, nursing home arrangements
Physiotherapist/OT	Allied Health	Mobility assessment, home safety assessment, equipment provision
Billing/Finance	Finance	Final bill preparation, payment processing, insurance claim submission

4. DISCHARGE PLANNING — EARLY INITIATION

Discharge planning must begin at admission, not when the patient is ready to leave. Proactive planning prevents avoidable delays and reduces hospital-acquired complications.

4.1 Admission Day Actions

- Admitting physician documents anticipated length of stay (ALOS) and estimated discharge date (EDD) in EMR.
- Social worker alerted if patient has identified social needs (elderly living alone, post-surgical home care, safeguarding concerns).
- Physiotherapy or OT referral made for patients with mobility, functional, or rehabilitation needs.
- Patient and family oriented on expected care pathway, timeline, and role in discharge preparation.
- Insurance pre-authorisation for extended stay initiated on day one for insured patients.

4.2 Daily Discharge Review

- Discharge criteria reviewed at each ward round; EDD updated in EMR if clinical course changes.
- MDT huddle held each morning to identify active barriers to discharge and assign actions.
- Outstanding investigations or specialist reviews tracked and actively expedited.
- Discharge lounge capacity checked; patients who meet criteria transferred to lounge to free bed.

4.3 72-Hour Discharge Planning Meeting

- For complex patients, a formal MDT discharge planning meeting is convened 72 hours before planned EDD.
- Attendees: physician, nurse, pharmacist, physiotherapist, social worker, and discharge coordinator.
- Meeting outcome documented in EMR; actions assigned with named leads and completion dates.

5. STANDARD DISCHARGE WORKFLOW

- Step 1.** Physician confirms patient meets all clinical discharge criteria; discharge order entered in EMR before 09:30.
- Step 2.** Ward nurse notified of discharge order; patient and family informed and preparation commenced.
- Step 3.** Pharmacist prepares discharge medications; reconciles with pre-admission and inpatient medications.
- Step 4.** Ward nurse conducts discharge education session covering diagnosis, medications, wound care, diet, activity restrictions, and warning signs.
- Step 5.** Patient receives written discharge instructions, medication list, and confirmed follow-up appointment details.
- Step 6.** Discharge summary completed by physician and electronically sent to GP within 24 hours.
- Step 7.** Billing team notified; final invoice prepared; insurance claim submission initiated.
- Step 8.** Patient or guardian signs discharge checklist confirming all education received and instructions understood.
- Step 9.** Transport arranged as appropriate (patient's own, ambulance, or hospital transport).
- Step 10.** Ward nurse documents discharge in EMR: condition at discharge, discharge time, and mode of transport.

NOTE: Target: Physician discharge order entered before 09:30 for planned discharges to achieve 11:00 bed clearance and support hospital flow management.

6. MEDICALLY COMPLEX & HIGH-RISK DISCHARGE

High-risk patients including the elderly, those with multiple comorbidities, post-ICU patients, complex wounds, and those with mental health needs require enhanced discharge planning to prevent avoidable readmission.

6.1 High-Risk Discharge Indicators

- Age greater than 75 with Clinical Frailty Scale score of 4 or above.
- Two or more hospital admissions in the previous 6 months for the same or related condition.
- Complex medication regime with 8 or more regular medications.
- Recent ICU stay or planned discharge on post-operative days 0 to 3.
- Active mental health condition or confirmed cognitive impairment.
- Unsafe home environment or absence of adequate social support network.
- Active wound requiring ongoing district nurse or community nursing care.
- New or significant change in functional status affecting activities of daily living.

6.2 Enhanced Discharge Actions

- Formal MDT discharge meeting convened at least 48 hours before planned discharge date.
- Home care package arranged in advance; community nurse referral submitted at least 24 hours before discharge.
- Specialist outpatient follow-up booked and confirmed before patient leaves ward; appointment card given.
- 24-hour hospital helpline number provided verbally and in writing for post-discharge queries.
- Post-discharge phone call scheduled and documented for 48 to 72 hours after discharge.
- GP notified by telephone for all high-risk discharges in addition to the standard written discharge summary.

7. AGAINST MEDICAL ADVICE (AMA) DISCHARGE

Where a patient insists on leaving hospital contrary to medical recommendation, staff must follow this protocol to protect both the patient's welfare and the hospital's duty of care obligations.

Step 1. Nurse identifies patient's intent to leave AMA; senior nurse and attending physician notified immediately.

Step 2. Physician meets with patient, explains clinical risks of early discharge, and documents the conversation fully in EMR.

Step 3. Mental capacity formally assessed; if patient lacks capacity, next of kin contacted and safeguarding protocol initiated.

Step 4. If patient has capacity and still wishes to leave, the AMA consent form is signed by patient (or documented as refused to sign, with witness).

Step 5. Discharge medications dispensed for immediate safety; urgent GP letter prepared and sent same day.

Step 6. Patient given hospital emergency contact number and explicitly advised to return if condition worsens.

Step 7. Incident report filed as Level 2 event; social worker alerted if safeguarding concerns are identified.

WARNING: Never physically restrain a patient with mental capacity from leaving the ward. Document all counselling attempts thoroughly. Restraint of a capacitous patient is unlawful.

8. DISCHARGE MEDICATION & EDUCATION

8.1 Pharmacist Medication Counselling

- Review every discharge medication with patient: name, dose, frequency, duration, and purpose explained in plain language.
- Highlight high-risk medications (anticoagulants, insulin, opioids, immunosuppressants): special instructions and monitoring requirements discussed.
- Address any drug-drug or drug-food interactions relevant to the patient's discharge prescription.
- Provide written medication information leaflets in the patient's preferred language.
- Confirm patient has adequate medication supply to cover the period until their first post-discharge follow-up.
- Identify and address any adherence concerns; simplify regime where clinically possible.

8.2 Patient Education Topics

- Understanding their diagnosis and what it means for daily life and long-term health.
- Wound care instructions and dressing change schedule with return date if applicable.
- Dietary restrictions, fluid intake guidance, and physical activity recommendations.
- Warning signs and symptoms that require urgent return to hospital or call for emergency services.
- Scheduled follow-up appointments and critical importance of attendance.
- Lifestyle modifications: smoking cessation, alcohol reduction, weight management, exercise plan.
- Self-monitoring instructions for patients with chronic conditions (blood pressure, blood glucose, weight).

9. DISCHARGE DOCUMENTATION

- **Discharge Summary:** Diagnosis, investigations, procedures, medications, and follow-up plan — sent electronically to GP within 24 hours.
- **Discharge Prescription:** Signed by physician; dispensed by pharmacist; copy filed in EMR.
- **Patient Instruction Sheet:** Plain language; includes red-flag symptoms, emergency contacts, and follow-up details.
- **Referral Letters:** Community nursing, physiotherapy, social services, and specialist follow-up as applicable.
- **Discharge Checklist:** Signed by patient and nurse confirming all elements of safe discharge completed.
- **Financial Clearance:** Billing team confirmation that payment or insurance clearance obtained before discharge.

Document	Responsibility	Timeframe
Discharge summary	Attending physician	Within 24 hours of discharge
Discharge prescription	Physician + Pharmacist	Before patient leaves ward
GP notification (high-risk)	Ward nurse / physician	Same day — telephone + letter
Community nursing referral	Discharge coordinator	At least 24 hrs before discharge
Patient instruction sheet	Ward nurse	Before patient leaves ward

10. POST-DISCHARGE FOLLOW-UP & EXCEPTIONS

10.1 Post-Discharge Call Programme

- All high-risk patients receive a structured phone call from the ward nurse or discharge coordinator within 48 to 72 hours.
- Call covers: medication adherence, symptoms, wound status, community service activation, and any concerns.
- Any concerns identified during the call are escalated to the GP or the direct readmission pathway is activated.
- Outcomes of all post-discharge calls documented in the EMR encounter record.

10.2 30-Day Readmission Review

- All readmissions within 30 days of discharge reviewed by the clinical governance team monthly.
- Root cause analysis performed: categories include premature discharge, inadequate education, medication error, or disease progression.
- Learnings incorporated into departmental improvement plans; outcomes reported quarterly to the clinical governance committee.

10.3 Exceptions

- Deceased patients: death certificate completed by physician within 24 hours; family sensitively supported; bereavement pathway activated.
- Transfers to other facilities: full transfer summary prepared; receiving facility confirmed available and prepared; ambulance or transport booked.
- Paediatric discharge: parent or guardian education mandatory; follow-up with paediatrician booked; red flag symptoms for children communicated.